



POLICY

Clinical Risk Assessment and Management (CRAM)

Scope (Staff):	NMHS Mental Health (MH) Staff: Clinical
Scope (Area):	All NMHS MH Areas

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1. Aim

To support a comprehensive and structured approach to clinical risk assessment, mitigation and promotion of safety and recovery for patients/consumers admitted/activated to NMHS Mental Health.

2. Background

Risk assessment and management in mental health care is inherently complex, requiring clinicians to navigate the challenges of balancing patient autonomy, therapeutic engagement, and duty of care. Risk is dynamic, shaped by individual, social, and systemic factors, often unfolding unpredictably. Clinicians make nuanced decisions in environments where uncertainty is the norm, recognising that risk cannot be eliminated but must be carefully assessed, documented and communicated. A shared understanding of risk is essential, with responsibility for safety shared between clinicians, patients, families, and other agencies, where appropriate. Through collaborative safety planning, risk management shifts from a solely clinical task to a partnership that promotes both safety and recovery of the individual and broader community where relevant.

The Clinical Risk Assessment and Management Policy provides an evidence-based framework to support clinicians in this challenging task. It aligns with key legislative and regulatory frameworks, including the ([Mental Health Act 2014 WA](#)), which mandates the Chief Psychiatrist's oversight of treatment and care standards, and the [National Safety and Quality Health Service \(NSQHS\) Standards](#), which set benchmarks for safe and effective care. Additionally, the [WA Department of Health Mental Health Policy Framework](#) and [WA Health Risk Management Policy](#) establish governance structures that inform clinical decision-making.

This policy is further informed by key **WA Department of Health (DoH) guidelines and best practice principles**, which provide structured approaches to risk assessment and management in clinical settings:

- [Principles and Best Practice for the Care of People Who May Be Suicidal](#), which emphasise evidence-based suicide prevention strategies, risk formulation, and recovery-oriented care.
- [Clinical Risk Management Guidelines](#), which outline systematic processes for identifying and responding to clinical risks in mental health care.
- [Guidelines for People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#).
- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#).
- which provide a framework for collaboratively developing safety plans with patients at risk of self-harm or suicide, ensuring care is individualised and supports autonomy.
- [MP 0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#), which mandates the use of Risk Assessment and Management Plan (RAMP) for documenting and managing clinical risks across WA Mental Health Services, ensuring consistency and accountability.

By embedding these principles into clinical practice, this policy aims to make risk assessment not simply a procedural task, but an integral part of patient centred, recovery-oriented mental health care. It promotes a culture of reflective practice, multidisciplinary collaboration, and ethical decision-making, supporting clinicians in making informed decisions in complex and high-risk clinical situations.

3. Service Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☒	Quality of Consumer/Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☒	Workforce Planning	☒
Medical Equipment	☐	Other	☒

4. Definitions

Clinical Handover	Clinical handover is effective, high-quality communication of relevant clinical information from a referring clinician to a receiving clinician when responsibility for patient/consumer care is transferred in accordance with the mandatory MP 0095 WA DoH Clinical Handover Policy under the WA DoH Clinical Governance, Safety and Quality Policy Framework .
iSOBAR	iSOBAR refers to the 6 key steps in the clinical handover process that specify minimum requirements for safe and effective clinical handover; I dentify; S ituation; O bservations; B ackground; A greed Plan / A ssessment and R eadback/ R isk/ R ecommendation (adapted from the MP 0095 WA DoH Clinical Handover Policy).
Clinical Risk	In mental health, clinical risk is defined as the likelihood of an event occurring, which may have harmful outcomes for the consumer or others.
Mental State Examination	A structured assessment of a patient's psychological functioning, including appearance, behavior, speech, mood, thought processes, cognition, perception, insight, and judgment. It helps clinicians evaluate the patient's current mental state and inform diagnosis, clinical risks and care.
Multi-disciplinary team (MDT)	Refers to a team including clinical and non-clinical roles from multiple disciplines who work together to deliver comprehensive care that deals with as many of the patient/consumer's health and other needs as possible.
Person-centred care	An approach to healthcare that prioritises the individual's preferences, needs, values, and active involvement in their care. It emphasises a collaborative relationship between the patient and healthcare providers, ensuring that care is respectful, responsive, and tailored to the person's unique circumstances.
RAMP	Risk Assessment and Management Plan (RAMP) is a mandatory clinical document on webPSOLIS that is used to document clinical risk in mental health services and forms part of the WA DoH Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation Guidelines .
Risk Assessment	The process of gathering information through communication, investigation, observation, and analysis to identify potential risks. It involves evaluating specific risk factors relevant to an individual and the circumstances under which they may arise.

Risk Formulation	A comprehensive and dynamic process of integrating information from multiple sources to understand and contextualise factors contributing to risk in a patient and articulate them in a structured manner to develop a tailored plan which addresses specific risks and promotes safety and recovery
Risk Management	The process of developing and implementing strategies to promote safety and recovery by addressing identified risks associated with a patient/consumer's mental health
Senior Mental Health Practitioner (Clinician)	Is a mental health practitioner with at least five years' experience in the treatment of persons with a mental illness.
Sexual Safety	Sexual Safety refers to being and feeling psychologically and physically safe, including being free of, and feeling safe from, behavior of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe. This includes sexual assault and harassment. It also extends to being spoken to using sexualised language or observing other people behaving in a sexually disinhibited manner, including nakedness and exposure or masturbation, being made to watch or shown pornographic images and lacking privacy and dignity when naked (Chief Psychiatrist's Guidelines for the Sexual Safety of Patient/Consumers of Mental Health Services (2020, P.16)).
Therapeutic alliance	A collaborative relationship between mental health professionals and patients/consumers, built on mutual respect, trust, and shared decision-making. It is characterised by a partnership in which both the patient and clinician work together to set goals, address challenges, and actively engage in the recovery process
Trauma-informed care	Trauma-informed care is an organisational and practice approach to delivering health and human services directed by a thorough understanding of the neurological, biological, psychological and social effects of trauma and its prevalence in society. It is a strengths-based framework that emphasises physical, psychological, emotional, social, cultural and sexual safety for people who have experienced trauma, their families and carers, and service providers.

5. Principles

5.1 General Principles

Risk assessment is a key component of mental health care in all settings. It is recognised that best practice involves a complex integration of individualised risk and protective factors for each consumer.

The following general principles should be considered when undertaking any clinical risk assessment:

- Risk is dynamic, can change rapidly and may be influenced by factors such as mental state, physical state, environmental changes and personal crises.
- Risks require regular review and assessment in response to the patient/consumer's changing presentation and circumstances.

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- Risk cannot be eliminated but mitigation strategies should be developed that are appropriate to the phase and focus of care.
 - Risk assessment tools cannot reliably be used to predict potential self-harm or suicide.
 - Risk assessment should always inform safety planning and contribute to clinical care and the wider assessment and planning of care needs.
 - Risk assessment and management must be culturally safe, ensuring that the diverse backgrounds, values, and beliefs of the patient/consumer are respected and integrated into the care planning and decision-making processes.
 - Decisions involving clinical risk must balance the safety and well-being of the individual with their right to autonomy, acknowledging that absolute safety cannot be guaranteed.
 - Building a therapeutic alliance and fostering a culturally safe, trusting, collaborative relationship between the clinician, the patient, and their support network is crucial. Shared responsibility for risk management should be emphasised, empowering the patient in the process.
 - Risk assessment and safety planning should be conducted collaboratively within the multi-disciplinary team and when required engagement with [Aboriginal Mental Health Support Services](#) and with the patient/consumer.
 - The individual's family, personal support persons and/or carers should be included when appropriate and in line with the patient/consumer's preferences.
 - If the patient/consumer is not involved in any aspect of risk management, the reasons for this should always be documented and revisited as appropriate.
 - Overall risk is a balance between short-term and long-term risks, and some strategies to mitigate one type of risk may affect other risks.
 - Clinicians should recognise deterioration in the patient's mental state and take appropriate actions, escalating care when necessary.
 - The response to deterioration must be person-centred, culturally safe, recovery-oriented, and in line with *Mental Health Act 2014 (WA)* – [Charter of Mental Health Care Principles](#).

6. Roles and Responsibilities

- **Clinicians:** Conduct risk assessments, involve the patient/consumer, and document decisions.
- **Multidisciplinary Team:** Collaborate on risk management and safety planning.
- **Patients/Consumers:** Engage in shared decision-making and safety planning.
- **Leadership:** Support a culture of risk awareness, accountability, and therapeutic engagement.

7. Clinical Risk Assessment, Formulation and Management

Clinical risk assessment, formulation and management are integral to health care. The format described below is intended to support and enhance current practice when assessing and managing clinical risks.

7.1 Risk Assessment

The risk assessment process is a critical component of patient care, involving a systematic,



multipronged approach. This includes establishing a therapeutic engagement with the person, utilising semi-structured assessments, structured risk-specific tools where relevant (e.g. Dynamic Appraisal of Situational Aggression (DASA), Historical Clinical Risk Management-20 (HCR-20v3) and RAMP), incorporating significant information from other sources including family/carers and other services, applying clinical judgment within a multidisciplinary framework to identify specific risks, risk factors, strengths, and supports that mitigate these risks.

These findings should be documented in the clinical notes and formalised in the RAMP, a mandatory State-wide Standardised Clinical Documentation (SSCD)

It should be noted that risk assessment tools, including the RAMP, and global stratification of risk into low, medium, and high should not be used to predict future risk or inform clinical decisions regarding admission, leave of absence, and discharge in isolation. The RAMP is a documentation tool, designed to capture the current risk profile and management plan, and clinical decisions should be based on comprehensive assessment, clinical judgment, and ongoing evaluation.

Risk is fluid and should be assessed in each clinical contact. Formal risk assessment documentation should be reviewed at regular intervals and in particular, when new information becomes available, when there is a change in the patient/consumer's clinical presentation or circumstances, transitions of care.

Risk assessments should take into consideration an individual's cognitive, emotional and social stages of development. This is particularly relevant when assessing youth and older adult populations.

Clinicians should be aware of the specific needs and vulnerabilities of Aboriginal Consumers, Culturally and Linguistically Diverse (CaLD) Consumers, Young People and LGBTQIA+ patients/consumers, including potential risks related to discrimination, stigma, and unique mental health challenges. Risk assessments should incorporate an understanding of gender identity and sexual orientation, ensuring that these factors do not contribute to risk or harm.

Provision should be made for patient/consumers who have limited cognitive ability or limited language or communication skills in accordance with the [MP 0051/17 DoH Language Services Policy](#) and the [DoH Language Services Guidelines](#).

Attempts must be made to establish a therapeutic and culturally safe engagement with patient/consumers. However, if risk of harm to the self or others is regarded as imminent and unacceptably high or potentially unmanageable, the risk assessment, escalation and management of risk would proceed with the resources available. The absence and disengagement – and indeed, the presence – of involvement should be recorded in all communications relevant to risk assessment.

Cultural factors should be considered in all aspects of risk assessment and formulation, acknowledging the patient/consumer's cultural identity, beliefs, and experiences in both risk identification and mitigation strategies.

A clinical risk assessment should make reference to the following elements.

7.1.1 Risk Factors

Consider factors that may increase a patient/consumer's risk/s such as, but not limited to, personality structure, substance use, social isolation, trauma history, age, gender,



psychiatric illness, cognitive impairment, cultural separation. These can be static (unchangeable e.g. history of self-harm) or dynamic (change quickly or slowly over time e.g. mood disturbance). Consider the impact of anticipated risk factors such as anniversaries, cultural obligations criminal proceedings, discharge, loss, stressful events, access to means and change in circumstances.

7.1.2 Risk Types:

Risk assessments should make reference to the nature and imminency of the potential harmful outcome/s, which may include risks to others (e.g. violence, aggression, harassment, stalking, sexual offending), risk to self (e.g. self-harm, suicide, misadventure, self-neglect, physical health, reputation, going missing) or risk from others (e.g. vulnerability, abuse, neglect, exploitation).

Consider the recency; severity; frequency and pattern of risk incidents or behaviours associated with the potential harmful outcomes.

7.1.3 Mental State Examination:

Consider the patient/consumer's appearance, level of engagement, behaviour, mood and affect including high levels of emotional distress thought processes and content including hopelessness, delusional beliefs, specific thought of harming self or others, perceptual disturbances such as command hallucination, cognitive impairment including disorientation, insight, capacity and judgement. A statement of intention from the patient/consumer is a clear indication of risk and should not be ignored but the absence of such a statement does not imply intent does not exist and should be explored through building a therapeutic alliance, trust and instilling of hope during assessment.

Consider motivation (why people take risks) and volition (how committed they are to act to understand the severity and acuity of risk). For example, if the person has thoughts to harm self or others these should be explored to establish whether they have considered how they will do this; the presence of a plan indicates a higher degree of risk including if they also have the means to carry out their plans.

7.1.4 Strengths and Protective factors

Strengths and protective factors should always be considered from the perspective of the patient/consumer. Consideration must be given to any factors with the capacity to prevent or reduce the likelihood of risks such as problem-solving skills, social, cultural and familial supports, engagement with service, insight and hope. Care should also be taken to equally consider any threats to protective factors. These factors make a significant contribution to the risk management plan.

7.1.5 Risk Formulation

Risk formulation aims to bring together the relevant factors to provide an explanatory framework for the patient/consumer's risks, how changes and interactions among risk factors, could elevate the identified risks or make them more imminent.

In formulating risk, clinicians should also take into account information and risks reported by carers and personal support persons or other third parties, in addition to the patient/consumer's strengths and protective factors which mitigate risks.



Risk formulation should be documented in the [RAMP\(SMHMR905\)](#) on webPSOLIS and be available in the physical or digital medical record.

Risk formulation can be structured using the 5Ps or components of the Risk Formulation Grid (see Appendix 1: Risk Formulation Grid)

Problem	The specific risk or concern that is being assessed, including presenting factors, signs, symptoms
Predisposing Factors	The factors that increase vulnerability to develop the risk behaviour(s): historical factors for example: • trauma (including intergenerational trauma) • early attachment • life adversity • past relationships • substance use • History of mental illness • Family history of suicide
Precipitating Factors	The factors that trigger the onset or exacerbate the risk behaviour(s): recent triggers, issues, for example: • significant life events • relapse of psychiatric condition • relationship breakdowns • financial crisis • criminal proceedings substance use
Perpetuating Factors	The factors that maintain the risk and prevent its resolution, for example: • personality factors • negative beliefs • relationships • psychosocial stressors • substance use • lack of engagement or non-concordance with treatment • social isolation
Protective Factors	The factors that prevent any deterioration in the risk. This can include any interventions in place, for example: • evidence of resilience

	• therapeutic alliance with treating clinicians • presence of hope • presence of connection to land, culture, spirituality • familial and social support • positive relationships
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7.2 Risk Management and Safety Planning

Risk assessments and risk formulation should inform the development of risk management and safety planning. Risk management plans should address in particular the modifiable risk factors, which are identified as precipitating and perpetuating factors. Where practicable, clinicians should work collaboratively with consumer/ carers and personal support persons, other support staff including peer support workers and aboriginal mental health liaison workers, to attend to the factors that matter to them, increase their sense of safety with individualised strategies and actions to mitigate /reduce the identified risk.

The risk assessment and management process must be trauma-informed, understanding that previous experiences of trauma can significantly influence a patient's mental health and response to care. Clinicians should remain sensitive to the potential for re-traumatisation and incorporate trauma-specific strategies into care planning.

Safety planning is a collaborative intervention which aims to enable a clinician to work jointly with a patient/consumer to identify tailored options to self-manage future vulnerable episodes e.g. suicidal thoughts and self-harm, and plan how to implement them. It should complement the risk assessment and formulation, drawing upon the patient/consumer's problem-solving abilities, strengths, protective factors and coping mechanisms.

In patients receiving care within forensic services, a shared decision-making approach to risk management may not always be possible or appropriate.

In instances where risk assessment presents with significant complexities, uncertainties, inconsistencies, ethical dilemmas or other challenges, clinicians should seek a multidisciplinary approach and involve senior clinicians to enable well-reasoned, balanced and shared decision-making. Clinicians should demonstrate the decision-making process in such situations through clear documentation of options considered including their merits, potential consequences and the clinical rationale in the notes.

Risk management plans should be documented in the [RAMP](#). Safety planning should be documented in the Treatment, Support and Discharge Plan ([TSDP](#)) or the WA Department of Health [Mental Health Consumer Safety Plan](#) (SMHMR932)

7.3 Review of Risk

Review of risk should be planned in advance with the involvement of patient/consumers, their carers and support persons wherever possible, and not be completed only in response to crisis. Open discussion with patients/consumers and their carers is likely to continue to improve outcomes. Efforts should be made to anticipate circumstances which may trigger a review (such as anniversaries). A risk assessment review should also be considered at other significant times for the consumer with increased risk.

For patient/consumers receiving community mental health care, assessment of risks should be embedded into every clinical contact and changes documented in clinical notes. As a minimum the RAMP should be formally reviewed and updated every 3 months as part of the multi-disciplinary team clinical reviews. Review of risks should also occur with every change to the phase of care and whenever deemed necessary, e.g. following any incident or crisis which would include deterioration in physical health, domestic abuse, suicide attempt or a violent incident.

For admitted patients/consumers, risk assessment is a continuous process and should be regularly documented in the clinical notes. This should be formally documented at a minimum in the RAMP every seven (7) days in the Multi-Disciplinary Team (MDT) meetings as well as when there is a change in circumstances, such as transition to another ward/service, change to MHA status, change to leave status and at the point of discharge.

When reviewing and updating risk assessments, clinicians should review the risk management and safety plan to appropriately address the changes in risk and/or protective factors.

8. Physical Health Risk Assessment

Physical illness can have a significant impact on a person's mental health and conversely mental illness can seriously impact on a person's physical health and potentially impact on risk: Clinicians should consider an individual's physical health as part of their overall risk assessment.

An escalation in physical health risks should be managed in accordance with the [NMHS MH Recognising and Responding to Acute Deterioration Physiological and Mental State Procedure](#).

It is recognised that physical health care monitoring is often provided in partnership with primary care in the community. Shared care and regular communication with patient/consumer's General Practitioner (GP) is crucial to managing physical health related risk factors and concerns, and should be done in accordance with the [NMHS MH Physical Healthcare Management Policy](#)

9. Sexual Safety

Sexual safety risks should be assessed and managed in line with the [Chief Psychiatrist's Sexual Safety Guidelines for the Sexual Safety of Consumers of Mental Health Services \(2020\)](#) and the [MH Sexual Safety in Inpatient Services Guideline](#). Risk assessments should include consideration of cultural, developmental, gender, and sexual orientation factors, ensuring that sexual safety strategies are individualised and relevant to each patient/consumer's unique needs.

Sexual safety risks should be actively identified as part of the overall risk assessment process.

Clinicians must assess factors such as:

- Vulnerability to sexual harm (e.g., mental state, history of trauma, current circumstances, cultural factors, gender identity, and sexual orientation).
- Risk of harm to others, including potential for sexual coercion or aggression.

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- Environmental factors that may impact sexual safety (e.g., hospital settings, ward configurations, and gender-specific needs).
 - LGBTQI+ specific vulnerabilities related to discrimination, stigma, and marginalisation.

A clear and individualised safety plan must be developed for each patient/consumer where sexual safety risks are identified, in collaboration with the individual, their support network, and the multidisciplinary team. Where appropriate, steps should be taken to protect the patient/consumer from potential harm, while also ensuring their autonomy and dignity are respected. Sexual safety planning should be informed by the patient's or consumer's cultural background, age, gender identity, and sexual orientation, ensuring culturally sensitive and developmentally appropriate interventions.

Clinicians should communicate and document any sexual safety concerns, including the implementation of any interventions, in the patient's care plan and across relevant communication channels.

Sexual safety risks and their management strategies should be regularly reviewed in the context of the patient's ongoing care, adjusting the safety plan as required in response to changing circumstances. Any incidents of sexual harm or breaches in safety should be thoroughly documented, investigated, and reported in accordance with the [NMHS Clinical Incident Management Policy](#) and [Chief Psychiatrist Reporting Notifiable Incidents](#) requirements.

10. Clinical Documentation

Risk assessment, formulation, and management should be documented in the RAMP, a mandatory SSCD tool located in webPSOLIS. Significant risks must also be entered into PSOLIS Risk Alerts in addition to the RAMP.

Risk reviews should be documented in the patient's clinical records to ensure accessibility for all clinicians involved in the patient's care. Progressive Risk Assessments (PRA) are used for concise documentation and communication of changes in risks within inpatient settings. However, caution is needed as these tools stratify risks; clinical judgment, based on multidisciplinary assessments, observed behaviors, mental state evaluations, and information from support networks, should guide decisions regarding leave status, specials, and discharge. Safety planning should be documented in the Treatment, Support and Discharge Plan (TSDP), and, where relevant, in the WA Department of Health Mental Health Patient Safety Plan (SMHMR932).

Communication of the assessment outcomes, safety plans, and management strategies should occur with the patient, other clinical teams or agencies supporting the individual and where appropriate family, carer, and Personal Support Person (PSP).

Clinical risk assessment and management planning documentation is governed by:

- [WA DoH Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation Guidelines](#)
- [NMHS MH Clinical Record Documentation Procedure](#)
- [NMHS MH Management of Clinical Alerts Policy](#)

11. Clinical Handover

The clinical handover process should identify patient/consumers who are at risk and provide information relevant to their management. All clinical handover processes must comply with the elements identified in the [MP 0095/18 Clinical Handover Policy](#) and follow the iSoBAR clinical handover process when communicating with members of the MDT.

12. Professional Education and Training

Education regarding clinical risk assessment and management is included in on-boarding and induction programs, orientation to work areas, annual mandatory training, clinical training and refresher training for clinicians working in inpatient and community MH services along with education relevant to the skills sets required for individual roles (e.g. physical health care screening and assessment)

All staff working in mental health services are required to comply with the [MHPHDS Mandatory Training Policy](#).

13. Compliance and Evaluation.

- The MHPHDS Safety, Quality and Performance is responsible for auditing and reporting to the MHPHDS Executive (SQPU Audit tools). The MHPHDS Safety, Quality, and Performance team is responsible for ensuring compliance through a comprehensive [clinical auditing program](#) and reporting to the MH leadership and Executive.
- Compliance with Mandatory Training is reported and monitored by service Leadership Committees.

Related internal policies, procedures, and guidelines

WA Department of Health

[WA DoH Mental Health Policy Framework](#)

[MP 0095/18 – Clinical Handover Policy](#)

[WA DoH Clinical Risk Management Guidelines](#)

[WA DoH Statewide Standardised Clinical Documentation \(SSCD\)](#)

[MP 0171/22 Recognising and Responding to Acute Deterioration Policy](#)

[WA DoH Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation Guidelines](#)

[MP 0051/17 DoH Language Services Policy](#) and [DoH Language Services Guidelines](#)

[MP 0006/16 Risk Management Policy](#)

NMHS

[NMHS Chaperone Policy](#)

NMHS Mental Health

[NMHS MH Case Management Policy](#)

[NMHS MH Community Mental Health Services Admission to Discharge Policy](#)

[NMHS MH Inpatient Mental Health Services Admission to Discharge Policy](#)

[NMHS MH Management of Clinical Alerts Policy](#)

[NMHS MH Clinical Record Documentation Procedure](#)

[NMHS MH Code Blue Medical Emergency Procedure - Inpatient MHS](#)

[NMHS MH Code Blue Medical Emergency Procedure - Community MHS](#)

[NMHS MH Medication: Management of High-Risk Medication](#)

[NMHS MH Medication: Prescribing Administration and Management for Inpatients Policy](#)

[NMHS MH Medication: Prescribing Administration and Management for Community MHS](#)

[NMHS MH Physical Healthcare Management Policy](#)

[NMHS MH Recognising and Responding to Acute Deterioration](#)

[Physiological and Mental State Procedure](#)

[NMHS MH Safe Use of Low Stimulus Environments Procedure](#)

[NMHS MH Sexual Safety in Inpatient Services Policy](#)

[NMHS MH Person of Concern Protocol](#)

[MHPHDS Mandatory Training Policy](#)

References

[Mental Health Act 2014](#)

[Chief Psychiatrist's Standards of Clinical Care](#)

[Chief Psychiatrist Standards and Guidelines](#)

[Chief Psychiatrist's Guidelines for the Sexual Safety of Patient/consumers of Mental Health Services](#)

[MHC Charter of Mental Health Care Principles](#)

[National Standards for Mental Health Services](#)

[National Consensus Statement: Essential Elements for Recognising and Responding to Clinical Deterioration;](#)

[National Consensus Statement Recognising and responding to deterioration in mental State;](#)

[A scoping review July 2014.](#)

[WA DoH Principles and Best Practice for the Care of People Who May Be Suicidal](#)

[WA DoH Principles and Best Practice for the Care of People who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Useful resources

[WA DoH Guiding Principles for Timely Administration of Medications](#)

[WA DoH Choice and Medication Patient/consumer Information](#)

[Carer's Recognition Act 2004](#)

[A Practical Guide for Working with Carers of People with a Mental Illness, Patient/consumer Carer Network Australia 2016](#)

[The Guardianship and Administration Act 1990](#)

[Victorian Government Office of the Chief Psychiatrist – White Paper: On the Principles of Mental Health Risk Assessment](#)

[National Institute for Health and Care Excellence – Self Harm: assessment, management and preventing recurrence](#)

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Policy Contact	Director of Medical Services MHPHDS				
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Approved by:	MH Policy Coordination Committee			Date:	19/03/2025
Endorsed by:	Executive Director MHPHDS			Date:	21/03/2025
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☒  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Seclusion and Bodily Restraint Reduction, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
06/11/2018	1.0	Clinical Nurse Consultant, Lower West Older Adult Program, NMHS MH	Initial endorsement
07/01/2019	1.1	A/Policy & Audit Officer	Minor Amendments: Updated DoH hyperlinks and policy links, updated NSQHS Standards to reflect V2, minor spelling and grammatical errors.
06/02/2021	1.2	A/Policy & Audit Officer	Minor Amendment: Link to rescinded DoH CRAM Policy removed from text and References. Link to superseding policy document added to References. Updated broken links.
07/10/2022	2.0	Director of Clinical Services MHPHDS	Scheduled review, major amendment incorporating changes in practice, policies and guidelines, ACSQHC Clinic Care Standards, audit and training program
19/03/ 2025	3.0	Director of Medical Services MHPHDS	Major review, update of clinical risk assessment, formulation and management in accordance with WA DoH Clinical Risk Management Guidelines, the National Safety and Quality Standards, Clinical Care Standards and the WA Safety Planning for Mental Health Consumers Policy.

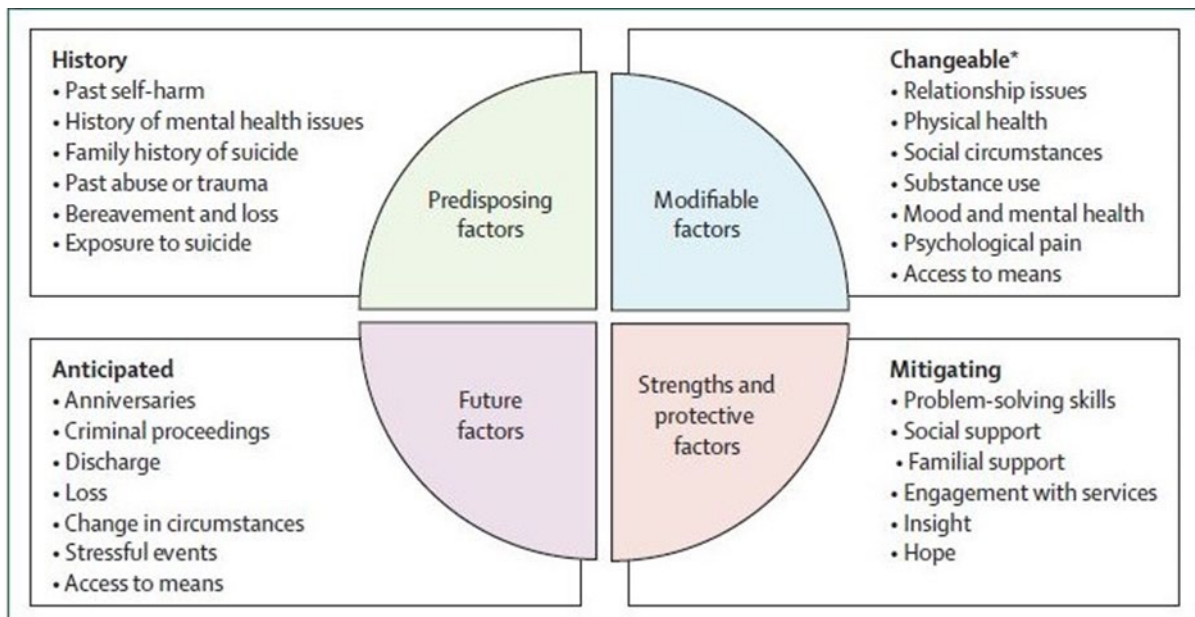
The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD 4133).

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14. Appendix 1 – Risk Formulation Grid



Source: Hawton et al 2022